National Cancer Control Programme (NCCP) 00279 · clinician draft

Cisplatin + etoposide with radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cisplatin Day 1 · high emetic risk

Active medicines: Cisplatin + Etoposide

Emetogenic risk: High

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1;80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) then daily Reference course; check interactions.
Ondansetron	16 mg orally	Day 1 Review QT risk and the whole multiday prescription.
Dexamethasone	12 mg orally Day 1;8 mg daily Days 2–4	Reference high-risk NCIS course Reconcile all steroids; no duplicate orders.
Olanzapine	5 mg orally daily Days 1–4	Reference high-risk NCIS course Review sedation and additive extrapyramidal risk with metoclopramide.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days The high-risk row does not specify an end day/PRN qualifier; prescriber specifies duration and suitability.

Additional phase-specific support

- Pre/post cisplatin hydration under local policy. Etoposide1000mL may supply prehydration; oral Day1 etoposide cannot.
- Source suggested posthydration:10mmol magnesium sulphate and20mmol potassium chloride in1000mL saline over2hours; electrolytes/volume must be checked against patient status and local infusion policy.
- Routine furosemide is not advised unless fluid overload; mannitol only according to local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00279 v8 · pNCCPpp1–2,5;NCISp4 · NCCP NCIS medical-oncology antiemetics v8

Sources checked 2026-11-02 · SACTCheck v0.84.30 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Days 2–3 · match actual drugs and route

Active medicines: Etoposide

Emetogenic risk: Route / schedule dependent

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCIS multiday guidance: give antiemetics appropriate to each day and for two days after. Continue the reconciled delayed cisplatin course; do not add another complete Day1 course.
- intravenous (IV) etoposide has low emetic risk. Oral etoposide requires the oral-route plan; split cisplatin means cisplatin is active on all three days. Pharmacy must issue one exact day-by-day prescription.
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by this National Cancer Control Programme (NCCP) document; additional prevention follows assessed risk/local policy.
- Assess eating/swallowing and refer for nutrition/pain support as needed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Only when carboplatinAUC5 substitution is prescribed

Active medicines: Carboplatin + Etoposide

Emetogenic risk: High carboplatin AUC≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1;80 mg orally Days 2–3	NCIS carboplatinAUC≥4 reference
Ondansetron	16 mg orally	Day 1
Dexamethasone	12 mg orally Day 1;8 mg daily Days 2–4	NCIS carboplatinAUC≥4 reference
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days Specify duration and suitability.

Additional phase-specific support

- This replaces the cisplatin support pathway; do not stack both high-risk regimens.
- Verify carboplatin renal dose independently; reconcile continuing etoposide route and multiday cover.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00279 v8 · pNCCPp1;NCISpp4–5 · NCCP NCIS medical-oncology antiemetics v8

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